

Arogya Radar

Gujarat state health radar — early warning for disease outbreaks, heat waves and medicine shortages. A detailed guide.

In one line: Arogya Radar watches the health of every area in Gujarat and warns officials about a disease outbreak days before it would normally be noticed — plus heat-wave risk, medicine shortages, and stock about to expire unused — and it does this mostly from data the government already collects.

The goal: make Gujarat the first state in India with real-time, block-level early warning for disease outbreaks and heat waves.



See it live — scan to open the working system

It covers all 33 districts of Gujarat and is live on Dahod today. Scan the code, or open:

arogya-radar-117722238113.asia-south1.run.app

The problem today

- Village and town health centres write their daily numbers on paper; officials see them only weekly or monthly.
- So outbreaks are noticed late, and medicines run out in one place while expiring unused in another.
- Every new system that asks busy staff to fill more forms tends to fail — the forms do not get filled.

The big idea — almost no new work for staff

Arogya Radar is different. It mostly **reads data the government already collects** — the e-Aushadhi medicine supply system, the TeCHO+ health platform used across all 33 districts, and IMD weather forecasts — and turns it into early warnings and clear actions. Only the smallest centres send anything new, and even then it is a 30-second voice note in Gujarati or a photo of the register they already keep. No new forms, no typing.

What Arogya Radar does — feature by feature

- **1. Early outbreak warning.** When several centres in one block report more fever or rash cases than usual, it raises a flag — using the same proven statistics that CDC and WHO use for disease surveillance, so an officer can see exactly why it fired. In Dahod it flagged a dengue-like cluster in Jhalod block, across 5 of 7 centres, about 9 days before a weekly report would have shown it.
- **2. A second warning that needs no reporting at all.** It can spot the same outbreak purely from how fast medicines are being used. When dengue kits, malaria tests and paracetamol are drawn down three times faster than normal, that is an outbreak — caught without anyone counting a single patient.

- **3. It looks ahead — a two-week forecast.** It projects where an outbreak is heading over the next 14 days and lists exactly which medicines to move in now, before the surge — so a district gets ahead of it instead of chasing it.
- **4. Heat-wave early warning.** Gujarat pioneered India's Heat Action Plans. The system shows the coming week's heat-index forecast, flags dangerous days, and checks that centres have the ORS and fluids ready.
- **5. Medicine shortages, expiry and redistribution.** It shows which centre will run out of what and when, finds stock about to expire unused, and suggests the transfers that fix both at once — for example, moving dengue test kits worth over a lakh from a centre in the west (where they would expire) to the outbreak in the east (where they are needed).
- **6. No blind spots, and accountability.** A centre that stops reporting is a dangerous blind spot, especially inside an area flagging an outbreak. The system names every silent centre and the last person who reported, so the district knows exactly who to call.
- **7. A closed loop — tracking the response.** Every alert can be marked Acknowledged, Team dispatched, or Resolved, with a time. So the state sees not just where the problems are, but which are being handled and how quickly — real accountability for outbreak response.
- **8. 30-second reporting for the last mile.** For small centres with nothing digital, staff send a short voice note in Gujarati, or a photo of the paper register. The AI turns it into a proper report; the worker just confirms with one tap.
- **9. A plain-language weekly summary.** One click turns the whole picture into a one-page brief for the Collector and CDHO, in plain Gujarati, Hindi or English.
- **10. The whole state on one screen.** Every one of Gujarat's 33 districts is on the map. Dahod is live today; the rest light up one block at a time as they come on board.

Where the information comes from

- **e-Aushadhi** (the state medicine supply system) — stock, consumption and expiry, to PHC level.
- **TeCHO+** — Gujarat's own frontline-health platform, already used across all 33 districts.
- **IHIP / IDSP** — the national disease-surveillance system, used as the baseline.
- **IMD** — the weather department's district temperature and heat forecast.
- **Field reports** — only for the last mile: a Gujarati voice note or a register photo. Four of these five sources need no data entry at all.

Rs 0 cost to run the trial	6 weeks length of the trial	~9 days earlier outbreak warning*	Rs 3+ lakh medicine waste found*
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* measured in a realistic test of Dahod district; a pilot will confirm the real numbers.

Is it safe? Data and trust

- It only **reads** information and advises. It cannot change any government record.
- It does **not** store patients' personal details — only daily totals.
- It is hosted in India; the **government owns all the data** and can stop any time.
- It is built to follow India's data-protection law (the DPDP Act, 2023).

The plan

- Start with a free 6-week trial in one block of **Dahod** district.

- Measure one clear result: how many days earlier an outbreak is caught, and how much medicine waste is prevented.
- If it works, expand across Gujarat — one district at a time.

How the 6-week pilot runs

- **Week 1** — connect to the block's existing e-Aushadhi data and brief a few staff. A few hours; no new systems to install.
- **Weeks 2–5** — the radar runs live; the block acts on its alerts.
- **Week 6** — measure the result and report to the district.

What we are asking for

We are asking only for permission to run that free trial, and a short meeting to show the working system — which already covers all 33 districts and is live on Dahod today.

Contact

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Arogya Radar · a working demonstration is ready to show now.